HALCYON RIDGE HEALTH SYSTEM | ENTERPRISE AI ENABLEMENT ADVISORY

Executive Issue Brief

AI demand is outpacing HRHS's ability to govern it, fund it, and staff it

Document	HRHS-AI-01	Version 2.1	November 2026 (supersedes v2.0)	Prepared by: Reginal Campbell, PMP
Audience and purpose	CEO, CFO, CIO/CTO, COO, CMO, and executive sponsors. Frame the decision and its resource constraints in five minutes.
Decision status	Advisory work product (hypothetical). No executive decision has been made. Decision requested at the ELT meeting of December 9, 2026 (scenario date).
Classification	FICTIONAL CASE STUDY. All organizational details, interviews, figures, dates, and findings are simulated scenario data, estimates, or labeled assumptions. No real engagement, approval, or result is claimed. Not a description of any real organization.

Executive assessment

> **BOTTOM LINE [J]**
>
> AI is already in use at HRHS, arriving department by department without shared controls. Every option must meet the same privacy, security, and contractual obligations. We provisionally recommend a governed, phased approach (Option B) because, beyond that shared floor, it builds reusable controls and shared capabilities that lower the cost and risk of each later use case. But the margin over a vendor-led approach (Option A) is narrow and depends on two things leadership controls: whether HRHS intends to pursue more than two AI use cases, and whether named engineering specialists can be released from ERP and EHR work. Approving Option B does not create engineering capacity. Without those commitments, the plan is not executable and Option A or a reduced scope is the better choice.

What is happening

- **Demand is fragmented [S].** Eleven use cases from seven sponsors; none compared for value or risk (F-01).
- **Two pilots run without enterprise review [S].** An ambient documentation pilot (P-01) processes patient information under vendor data-use and retention terms Privacy has not reviewed (an unresolved risk; no disclosure or violation has been identified); a denial-prediction pilot (P-02) has no success measure (F-04, F-05).
- **Capacity, especially specialist capacity, is the binding constraint [S].** About 6,800 of 85,000 discretionary IT hours are uncommitted. Option B needs about 14,000, including 4,600 hours of ERP data, integration, security, and architecture specialists who are committed to ERP and EHR programs (F-08).

Why act this budget cycle

Pressure	Implication
Margin 1.2% vs 3.0%	Supply spend ($640M) and denial write-offs ($38M/yr) are large levers, but only a fraction is addressable by AI and benefits lag costs (Document 03).
Unresolved PHI vendor risk	HIPAA obligations apply to P-01 now, and the vendor terms are unverified. The contract auto-renews January 31, 2027 with an expansion option, so review must precede renewal (R-01).
Budget lock	FY2027 budgets close in December. A provisional AI envelope of up to $4.5M [A-01] will be reallocated if not justified.

Options and financial exposure [E]

Option	FY2027 cash	3-year cost	Main tradeoff
A. Vendor-Led	$2.55M	$6.35M	Fastest, lowest cost; controls applied product by product; most vendor dependency.
B. Governed, Phased	$3.75M	$7.75M*	Reusable controls pay off only with a pipeline; ~$1.4M more than A over 3 years; needs specialist release.
C. Platform-First	$7.9M	$12.3M	Strongest foundation; exceeds envelope and capacity in FY2027.

**Includes Wave 2 (revenue cycle), which requires separate authorization. Quantified benefits do not repay any option within three years in the low case; see Document 03.*

Leadership action requested December 9, 2026

ID	Decision	Approver
D-01	Select the approach (recommended, provisionally: Option B).	CEO, on ELT advice
D-02	Authorize up to $3.75M in two tranches ($1.5M now; $2.25M at Day 90 on evidence); $0.75M contingency held by CFO.	CFO
D-03	Charter lean AI governance and adopt interim policy and controls.	CEO
D-04	Bring P-01 and P-02 under governance within 30 days; replace P-01 auto-renewal with a 90-day extension.	CMO (P-01), CFO (P-02)
D-05	Submit provisional named specialist allocations by Day 15 and validate them against ERP/EHR release calendars by Day 20; if any are unavailable in the delivery window, invoke Document 08.	CIO/CTO, with ERP and EHR sponsors

D-03 and D-04 reduce unresolved risk at little cost and should proceed even if D-01 is deferred.